

Ontario's transport gap

Ontario has a patient transportation **industry**. It does not have a patient transportation **system**. Five separate networks move the same patients on different days of the week, and none of them can see the others.

01 What the survey found

Six findings drive every recommendation that follows. Each is sourced; each carries a confidence rating in the accompanying register.

FINDING 01 The waste is documented and large The only population-level study of Ontario inter-facility transfers found ~400,000 a year costing ~\$283M, with 80% routine and non-urgent — largely carried by fully-equipped 911 ambulances. The most common trip: 10.5 km to a cardiologist or a dialysis chair.	FINDING 02 There is no payer OHIP does not cover non-emergency transport. Emergency ambulance carries a regulated \$45 co-payment; non-urgent stretcher transport is unregulated in price and uncovered, with no exemption available even on social assistance.
FINDING 03 The US playbook does not transfer American NEMT software is built on Medicaid managed-care brokers that supply volume, prior authorization and a claims rail. Ontario has neither a broker layer nor a claims rail. Importing that architecture builds a product with nothing to plug into.	FINDING 04 The buyer is fragmented — but real Hospitals, LTC operators, community agencies and regional renal programs each hold a piece. Two are single-payer pools: NIHB medical transportation in Ontario at \$166.0M and growing ~20% a year, and the Northern Health Travel Grant.

FINDING 05

Supply is consolidating

Transdev/Voyago — 600+ medical attendants, positioned as Canada's largest NEPT brand — has been acquiring regional operators. A neutral coordination layer gets harder to build every year consolidation runs.

FINDING 06

A vendor just collapsed under this market

Quebec ePCR vendor **Prehos** gave notice on 8 June 2026 and ceased operating 7 July, owing ~\$10M. ~22 **Ontario paramedic services** were displaced on under a month's notice; some reverted to paper. Prepaid fees became unsecured claims and one service had to buy new laptops. The displaced migrated **cloud to cloud** — two vendors to replace one.

FINDING 07

The obvious product is the wrong one

Per-vehicle dispatch SaaS against Ontario's addressable fleet tops out near **\$3.4–5.8M ARR at total penetration**. The value is not in selling consoles to operators — it is in owning the allocation decision and the settlement rail.

02

The misallocation, in one bar

This is the finding the entire opportunity rests on. Four in five inter-facility transfers are clinically routine, yet the default vehicle is an emergency ambulance with a paramedic crew — because no one is making the allocation decision on purpose.

ONTARIO INTER-FACILITY TRANSFERS BY CLINICAL URGENCY

~400,000 transfers a year · about 1,000 a day



Source: population-level study of Ontario inter-facility transfers (ICES / U of T / Ornge / Sunnybrook), published in *Healthcare Policy*. Baseline year 2009 – see the caveat in §10. Scaled to 2026 the volume estimate is 500,000–690,000 transfers a year.

~\$2B Ontario land ambulance spend per year (province funds ~\$1B at a 50% cost share)	300,000 Hours a year lost to offload delay in Toronto alone; Ottawa 93,686	17.0% Of Canadian hospital days are alternate-level-of-care – beds held by patients ready to leave	10% Of dialysis patients miss at least one session a month; 35% miss one per quarter
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03 Five silos, no system

Ontario already proved it can coordinate patient transport — for the acute, high-severity, provincially-funded minority. CritiCall's Repatriation Tool has served every acute hospital since 2014, and Ornge moved 23,725 patient legs in 2024/25 across a million square kilometres.

The routine 80% never got that treatment, because no ministry owns it.

NETWORK	OPERATED BY	PAID BY	COORDINATION
911 land ambulance	~50 municipal paramedic services	Province 50% / municipal 50%	CENTRAL DISPATCH
Critical care & air	Ornge · CritiCall for placement	Province	PROVINCIAL
Non-urgent patient transfer	Dozens of private operators	Hospital · LTC · the patient	NONE
Specialized / para transit	Municipal transit (Wheel-Trans et al.)	Municipal + fare	MUNICIPAL ONLY
Community assisted transport	Support agencies, volunteer drivers	Ontario Health + co-pay	AGENCY BY AGENCY

The regulatory hole

Non-urgent patient transfer sits **outside the Ambulance Act**: no licensing, no vehicle standard, no crew standard, no price ceiling. A coroner has called for regulation since 1995; a 2011 bill died when the legislature dissolved; Ontario's Ombudsman has called paramedic oversight "complicated, overburdened, and wholly inadequate."

Twenty-five years of failed attempts say **do not bet on regulation arriving**. Build a credentialing and audit spine anyway — so that if it lands, you are the reference implementation rather than a casualty.

04

Who pays — the crux

Emergency transport has one funding structure and a regulated price. Everything else has neither.

Emergency (911)

REGULATED · ~\$2B/YR

- Province 50% / municipality 50%
- Patient co-payment \$45
- \$240 if deemed not medically necessary
- Statutory exemptions exist — though "many people are not aware" of them

Non-urgent transport

UNREGULATED · NO OHIP COVERAGE

- Hospital budget · LTC home budget
- Patient and family, out of pocket, no exemption
- NIHB — \$166.0M in Ontario (2023–24)
- Northern Health Travel Grant — reimbursed after the fact
- WSIB · private insurers · charities
- Ontario Health atHome and community agencies + co-pay
- Municipal paratransit + fare

Three properties make this the business model, not just context

"Medically necessary" is undefined in policy, legislation or regulation. Hospitals have told the Patient Ombudsman that needing a stretcher does not by itself establish medical necessity. The most consequential financial decision in the trip has no decision support behind it.

Nobody sees total cost. One patient can consume a municipal paratransit trip on Monday, a hospital-funded transfer on Wednesday and an NIHB-funded trip on Friday. No system reconciles this.

Eligibility goes unclaimed. The Northern grant requires the patient to front the money and file afterward — roughly 200,000 reimbursements processed in 2023–24, at \$0.41/km. Automated eligibility and split settlement is genuinely unbuilt infrastructure.

The most valuable regulatory fact here

Ontario's **OADS v4.0** — the Ambulance Documentation Standards, mandatory since 2 September 2025 under O. Reg. 257/00 — is the compliance moat that protects the incumbent ePCR vendors. It binds Ambulance Service Operators, paramedics and Base Hospitals.

Because non-urgent patient transfer sits **outside the Ambulance Act, it also sits outside OADS**. A transport product that never becomes the 911 record of care does not have to cross that moat. The same absence of regulation that makes this market chaotic to operate in makes it **cheap to enter with software**.

⚠ The exemption is conditional. Right-sizing triage decides whether a patient needs an ambulance — the nearer that moves to diverting emergency resources, the nearer it moves back to the regulated boundary. **Display and record clinician decisions; never derive them.** And confirm the exemption with a legal opinion before it carries weight in a pitch.

05

Demand is compounding

Three forces push the same direction, and one of them is being deliberately created by provincial policy.

Demographics

Ontario had **3.1 million people aged 65+ in 2025** — 18.9% of the population, heading to 4.6 million by 2051, with every baby boomer past 65 by 2031. That cohort has grown $\sim 1.7\times$ since the transfer study was published, against $\sim 1.25\times$ total population growth.

Recurring treatment travel

More than **11,000 Ontarians are on dialysis**, roughly 78% of them in-centre. In-centre means three trips a week, indefinitely — no other patient group in health care carries that transport dependency. It generates about 1.33 million round trips a year, every one of them known in advance.

Policy is manufacturing trips

Ontario funded **57 new community surgical and diagnostic centres** at \$155M over two years, including 35 for MRI and CT serving a claimed 800,000+ patients. Every one of them decouples a procedure from a hospital — and creates a journey that did not previously exist.

Non-Insured Health Benefits, fiscal year

\$114.8M

2021–22

\$140.3M

2022–23 · +22.2%

\$166.0M

2023–24 · +18.3%

Second-highest of any province after Manitoba. A single federal payer, already contracting, compounding at roughly 20% a year – the most attractive payer structure in the Ontario market.

06

Who else is here

Four groups compete for adjacent ground. None occupies the intersection that matters.

CATEGORY	WHO	THREAT	WHY
US NEMT SaaS	RouteGenie · TobiCloud · Bambi · TripMaster · MediRoutes	LOW	Architecturally coupled to Medicaid brokers — RouteGenie's headline feature is 25+ broker connections. None of those rails exist here. \$50–\$200+/vehicle/mo.
Hospital orchestration	Ryde Central	HIGH	Closest analogue to the right product: hospital ↔ provider automation, EHR pull, CAD integration. US-oriented, no confirmed Ontario presence — a fast follower could land here first.
Operator portals	goPatient (Transdev) · Voyago booking	STRUCTURAL	Every consolidator portal that locks in a hospital shrinks the neutral market. This is the clock on the opportunity.
Rideshare health	Uber Health — live in Ontario	HIGH, AMBULATORY ONLY	No app needed, multilingual, Hope Air partnership. Cannot do stretcher, bariatric, oxygen, or cognitive-impairment escort. Takes the easy top, leaves the high-acuity margin.
Ontario ePCR incumbents	ImageTrend · ESO/Interdev iMedic	ADJACENT	Not transport competitors, but they hold the paramedic-service and hospital relationships. ImageTrend crossed the OADS moat in March 2025; ESO's iMedic ships CADLink real-time dispatch. Likeliest parties to extend sideways.
Canadian transit tech	Spare Labs · Pantonium · Blaise	MEDIUM	Strong optimisation, municipal channel, Canadian data residency — but transit-agency shaped, not clinically shaped. Partner rather than fight.
Traumasoftware (corrected)	US private-EMS operations platform	CATEGORY PROOF	Not a US NEMT point solution: CAD, ePCR, billing, scheduling and fleet in one, founded 2006, ~200 customers, aimed at private EMS and NEMT operators. NEMSIS, not OADS; no Canadian footprint evidenced. Its scale proves the integrated category supports a real business — in a market that has a broker rail.

The whitespace, stated plainly

Nobody occupies the intersection of **clinical acuity triage × multi-provider neutrality × multi-payer settlement × Ontario regulatory fit**.

US SaaS has neutrality and no clinical or payer fit. Uber Health has scale and no acuity. Operator portals have clinical fit and no neutrality. Transit tech has optimisation and no clinical model. CritiCall has coordination and no reach into the routine 80%.

07 Ottawa

Ottawa is the better first market in Ontario, and the reason is counter-intuitive: **its ambulance crisis is largely over**.

AMBULANCE OFFLOAD DELAY · OTTAWA HOSPITALS

2022 → 2025, minutes

HOSPITAL	2022	2025
Montfort	224 min	53 min
Queensway Carleton	168 min	53 min
TOH General	163 min	58 min
TOH Civic	136 min	47 min
CHEO	39 min	36 min

"Level zero" – no ambulance available anywhere in the city – fell to **866 minutes in 2025, down 92% from 11,372 in 2024**. The service still missed its CTAS 1 target (70.4% against 75%), so it is not solved – but the availability crisis is no longer the live wound it is in Toronto.

Do not lead an Ottawa pitch with ambulance hours saved

That argument was already won by someone else. Repeating it to Ottawa Paramedic Service reads as not having done the homework. Lead instead with **hospital discharge flow** and **recurring treatment travel** — both unresolved.

900+

Dialysis patients across 10 TOH sites — one of Canada's largest nephrology programs

~109,000

Estimated in-centre round trips a year, on fixed schedules to known addresses

1 in 5

Beds at TOH Civic held by ALC patients; 1 in 6 at Montfort and Queensway Carleton

878,000

Para Transpo trips, Nov 2024–Oct 2025, up ~3.6%

The dialysis anchor

The Ottawa Hospital's Regional Nephrology Program runs three in-centre units, four satellites and a home program across ten sites, serving Ottawa and Eastern Ontario. Applying the provincial in-centre share gives roughly **700 patients × 3 sessions a week ≈ 109,000 round trips a year** — under a single clinical owner.

That is the most concentrated, most poolable, most contractable block of recurring medical transport demand this survey found anywhere in Ontario. And it sits in a city of ~1.1M rather than 3M, so one operator can plausibly hold a meaningful share.

Bilingual service is a contract requirement in practice

Ottawa is a designated area under the **French Language Services Act**, and Montfort is the province's francophone teaching hospital. Be precise about the mechanism, though: **the Act binds government and designated public-service agencies, not a private transport operator directly** — a vendor's obligation flows through its contract with a designated entity.

The practical expectation holds — win Montfort or TOH work and expect French in the contract — but it is **contractually near-certain rather than statutorily automatic**, and the designation scope for health transport is still unconfirmed. Most transfer operators do not document bilingual capability, and a product built on the EMSS core inherits **EN/FR with loadable language packs already built**, so it remains a differentiator at near-zero marginal cost.

Competition

Thinner than Toronto, but the strongest player is the consolidator. **Priority Patient Transfer Service** — acquired by Transdev/Voyago — runs **goPatient**, a booking portal offered exclusively to contract hospitals, so switching costs are already installed. **Ontario Patient Transfer** operates one of its three super bases here, plus satellites at Winchester and Cornwall.

08

Toronto

The largest pool in the province and the hardest to win. Toronto is where the survey's central thesis is most true and least ownable.

700+ Hours a day paramedics spend in hospital – about 60% of total service time (2024)	350,000 Emergency calls in 2024, forecast to grow ~3% a year	42% Of cases took longer than 30 minutes to offload (2023)	3.54M Wheel-Trans trips in 2024, 42,000+ registrants, ~9,700 riders a day
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Where Ottawa fixed its problem, Toronto's is unresolved and structural. Roughly **255,000 hours a year of paramedic capacity** is absorbed by hospital handover, on a 2025 budget of \$372.3M. Every argument in §01 about non-urgent transfers consuming emergency capacity applies here at full strength — the right-sizing surface has its strongest possible audience in Toronto.

Toronto also holds the densest cluster of academic and community hospitals in Canada. Academic centres generate inter-facility transfers structurally: patients move between sites for imaging, procedures, consults and repatriation. Short distances, high frequency.

Why it is still the wrong place to start

The competitive field is the densest in Ontario — Voyago, RNR, Encore, Swift Med Care, Health Lync, Ontime, MD Transfer. Price competition in an unregulated market is brutal.

Uber Health is live in Toronto and Brampton. The ambulatory tier — the easiest, highest-margin trips — is actively being taken by a company with better logistics and no app requirement for riders. What remains is stretcher, bariatric, oxygen and escort work: harder, slower, more expensive to staff.

No single buyer holds enough demand. Toronto's dialysis population is larger in absolute terms but split across many hospital corporations, so no one contract anchors a fleet the way TOH's 900-patient program does.

	OTTAWA	TORONTO
Offload crisis	Largely resolved — don't lead with it	Severe and live — strongest triage audience
Dialysis anchor	900+ patients, one program	Larger, but fragmented
Competitive density	Moderate; Transdev/Voyago incumbent	Highest in Ontario, plus Uber Health
Language	Bilingual required (FLSA, Montfort)	Multilingual, not statutorily bound
Verdict	START HERE	EXPAND HERE

09 Seven product surfaces

Ranked by defensibility against evidence of pain.

S1	Acuity triage & right-sizing Decide per trip: 911 ambulance, stretcher van, wheelchair van, sedan, volunteer driver — or no trip at all. Where the documented waste lives, and where "medically necessary" is currently decided with no decision support.	HIGHEST VALUE
S2	Recurring-trip optimiser Standing orders for dialysis, oncology, rehab and wound care; pooling across patients and providers; automatic re-optimisation on schedule change.	BEST WEDGE

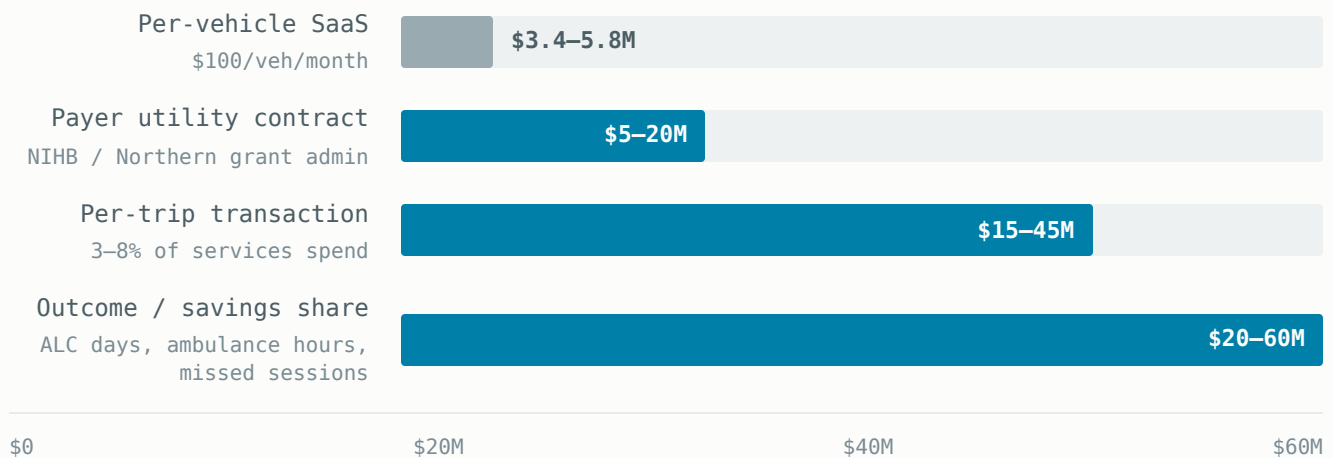
S3	Neutral booking exchange One PHIPA-safe request, fanned to a multi-provider market with real acceptance and ETA. Directly contests operator-captive portals — and has the shortest window before consolidation closes it.	TIME-LIMITED
S4	Discharge-flow integration Predict discharge readiness from hospital signals, pre-book transport, tie vehicle arrival to bed release. Sells against ALC days, which hospitals already measure.	NEAR-TERM
S5	Eligibility & split settlement Determine who pays which fraction — NIHB, the Northern grant, hospital, LTC, insurer, patient — and settle it. Unbuilt. The deepest moat and the slowest build.	LONG MOAT
S6	Northern & Indigenous journeys Multi-leg land and air, grant pre-authorisation instead of after-the-fact reimbursement, Ornge and Hope Air handoffs. A federal payer growing 20% a year, and the strongest equity case.	SINGLE PAYER
S7	Quality & credentialing spine Crew credentials, vehicle standards, incident reporting and outcome tracking in a sector with none. Low near-term revenue; the highest-leverage asset if regulation ever lands.	OPTION VALUE

10 The SaaS trap

The obvious business is to sell dispatch software per vehicle. Sized honestly against Ontario's entire eligible fleet — roughly 3,000–5,000 vehicles across patient transfer, land ambulance, paratransit and community fleets — it does not clear the bar.

ONTARIO REVENUE CEILING BY BUSINESS MODEL

Upper bound at full penetration, annual



Bars show the upper bound of each estimated range. Per-vehicle SaaS is shown in grey as the non-viable case. Realistic penetration against entrenched incumbents puts the SaaS model at \$0.7–1.8M ARR. Full derivation and every assumption in `docs/02-opportunity-model.md`.

The rule this implies

Price against avoided cost, never against seats. Every viable model here requires sitting in either the allocation decision or the flow of funds. A product that sits in neither is a \$4M business — and does not justify the clinical and regulatory work this domain demands.

11

What carries over from EMSS

Corrected. The first version of this section assumed EMSS meant a dispatch/CAD system and listed dispatch logic, AVL and crew scheduling as assets that carried over. The codebase has now been read. **None of those exist**, and every row of the original table was wrong.

EMSS is AidPost — an offline-first PWA for casualty care and transport documentation. ~45,500 lines across 195 files. Responders capture identity, injuries on an anatomical body chart, vitals and treatments, then hand over to a hospital as an HL7 FHIR R4 bundle. Multilingual with right-to-left support.

ASSET	WHERE	WHY IT MATTERS HERE
Ontario Health EHR integration	core/ehr/ontario.ts packages/ehr-gateway	Provincial Client Registry <i>Patient/\$match</i> , OHIP identifier systems, ATNA AuditEvent on every access, ONE ID auth, mTLS. This survey recommended building FHIR-native against the provincial registries — it is already built .
Conflict-aware sync	core/sync/oplog.ts	Append-only ops with Lamport clocks folded in total canonical order — byte-identical state across replicas regardless of arrival order, conflicts reported rather than silently lost. Directly reusable for driver devices that go offline in Northern Ontario.
Multi-tenant backend	packages/sync-service	Tenant context and store, OIDC, ops and blob stores, retention, admin and EHR audit stores, Postgres and SQLite.
PHIPA primitives	app + core	AES-256-GCM vault, RBAC-lite, step-up PIN, tamper-evident audit log, minimum-necessary masking with timed audited break-glass, propagating erasure.
i18n with RTL	src/i18n.tsx	Loadable JSON language packs — a deployment adds a language with no code release.

The thesis does not transfer — and that is the important part

AidPost's differentiator, sharpened across four survey passes, is **"no backend, no sync server, no vendor relationship."** Its own fourth pass retired "works offline" as a headline because the incumbents do offline capture too; what remains distinctive is the *absence* of a server, an account and a vendor.

A transport coordination layer cannot be built that way. Matching a request to a provider, settling across payers and holding shared fleet state are inherently multi-party and networked. A booking exchange requires a server by definition — precisely the shape AidPost defines itself against.

Reuse the engine. Do not reuse the pitch. Dispatch, ETA prediction, pooling, fleet management, booking, eligibility and settlement are all new work. That is most of the product.

AidPost's own commercialization file scores Official EMS at ★ fit, ★ accessibility, heavy regulatory load, 12–24+ months — "the biggest prize but a fortress" — and recommends the humanitarian path instead. A patient transportation product is not that fortress: outside the Ambulance Act, outside OADS, logistics rather than SaMD. But it is still Ontario health-system selling, and it is a second front for the same team. Those are resourcing decisions, not research findings.

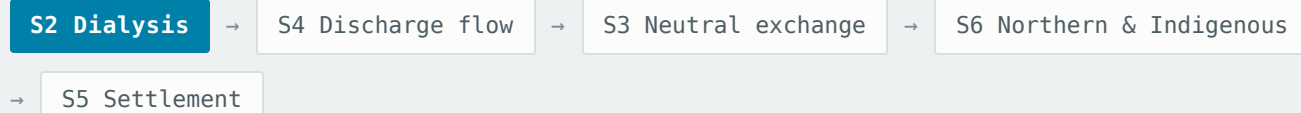
Recommendation

Do not build a dispatch platform. Build the allocation and settlement layer
 Ontario does not have — starting from a single wedge with a real budget holder
 and a measurable outcome.

RECOMMENDED BEACHHEAD

Recurring dialysis transport, sold to regional renal programs

1. **Frequency.** Three trips a week per patient, indefinitely. ~11,000 dialysis patients, ~78% in-centre — about 1.33 million round trips a year.
2. **Schedulability.** Treatment slots are fixed and known in advance. This is the most poolable demand in the entire health system.
3. **An identified buyer.** Regional renal programs under Ontario Health, with an existing performance-reporting relationship.
4. **A metric already tracked.** Missed treatments — 10% monthly, 35% quarterly — with published morbidity consequences. Value provable in one quarter.
5. **A natural expansion path.** Oncology and rehab share the recurring structure, and dialysis patients also generate discharge and inter-facility trips — walking you into S4 and S3 through a door you already hold.



One caution from the EMSS survey

The Prehos collapse is the closest thing this market has to a natural experiment, and it cuts against architectural pitches. Twenty-two Ontario services were displaced by a **commercial** failure — not an outage — and the one documented migration went **cloud to cloud**, needing two vendors to replace one.

Sell the outcome, not the architecture. For the dialysis wedge that means missed treatments avoided — a number the buyer already tracks and already answers for. Data ownership and continuity are a strong second argument and a poor first one.

Two decisions the research cannot make

Whether this product is a **coordination layer**, an **operator**, or a **payer-side utility** — and which wedge gets funded first. Both are the team's calls. Everything above is the input to them.

Method and its limits. Research conducted August 2026. Direct document retrieval was blocked by network egress policy for every primary-source domain attempted – including ICES, PubMed Central, ontario.ca, CIHI and the Ombudsman. Figures were therefore reconstructed from search-result summaries of those documents, not from the documents themselves.

That is adequate for shaping product strategy. It is **not** adequate for a business case, a board deck, or anything a third party will rely on. Every figure carries a confidence rating in the source register, together with a prioritised verification list – led by refreshing the 2009 transfer study, obtaining real Ontario patient-transfer pricing, and confirming what share of dialysis patients actually use assisted transport.

Revision 2. Reconciled against the EMSS repository (product name AidPost), reviewed read-only at commit **a8eb1d9**. That repository carries its own four-pass Canadian market survey using adversarial three-vote claim verification – better-evidenced than this document, and hitting the same egress constraint. Findings drawn from it are marked in the full survey, and **where the two disagree, theirs wins**. Section 09 of this survey was wrong and has been rewritten against the actual code.

Full survey, sizing model, source register and reuse assessment: **docs/01-market-survey.md · docs/02-opportunity-model.md · docs/03-source-register.md · docs/04-emss-reuse-assessment.md**